

Testosterone Replacement Clinical Reference

Male TRT - testosterone cypionate · Provider use only

KORB Health Medical Texas PA · Testosterone Replacement Clinical Reference v1.1 · effective 2026-09-16 · built 2026-09-17

Testosterone cypionate 200 mg/mL for male testosterone replacement. This document is the protocol. The TRT Provider Tool computes the visit, refill and lab dates for a specific patient and produces the same prescribing fields shown here; both read this data file, so they cannot disagree.

Supersedes. Nothing. This is the first standalone TRT reference; before it, the protocol existed only inside the TRT Provider Tool.

How TRT works at KORB

Testosterone Cypionate 200mg per mL inj (commercial generic). One presentation, one concentration: 200 mg/mL in a 10 mL vial (2000 mg).

This is a COMMERCIAL product. It is dispensed by a compounding pharmacy, which is a different thing, and the Tebra entry is an ordinary standard prescription - the drug is selected from the drop-down and there is no reason for compounding, because nothing is compounded.

Testosterone is a Schedule III controlled substance. Refills run from the PMP last fill date, not the visit date, and nothing is dispensed or scheduled beyond 90 days.

Storage: Room temperature. Do NOT refrigerate at any point, including in transit. Discard 90 days after first use.

Before you start - the baseline gate

Confirm ALL of the following before the first prescription. If any is not met, defer therapy and repeat testing or refer - hematology, urology or endocrinology as the finding indicates.

- Hematocrit below 52%.
- PSA within the age-adjusted normal range for the patient (see Prostate safety).
- Normal liver and kidney function, if tested.
- No absolute contraindication present.
- Symptoms and laboratory findings align. Testosterone deficiency is diagnosed on both, not on a number alone.
- FERTILITY HAS BEEN ASKED ABOUT AND THE ANSWER DOCUMENTED. See the next section.

Abnormal labs must be documented in Tebra with an interpretation and a plan, not merely filed. Where therapy proceeds despite a borderline finding, the note must carry the rationale.

Fertility - ask before you prescribe

Testosterone suppresses spermatogenesis. Counsel every patient who may want children before starting, and document the discussion and the patient preference in the chart.

- An active desire for fertility WITHOUT that support is an absolute contraindication, not a relative one. Refer rather than treat.
- Enclomiphene 25 mg orally daily, or hCG 500 IU two to three times weekly, are the adjuncts that preserve fertility alongside therapy. KORB DOES NOT CURRENTLY OFFER EITHER. They are recorded here so the option is discussed and referred rather than overlooked, and they may be offered if a need arises.

ASK ABOUT FERTILITY BEFORE YOU PRESCRIBE. Testosterone suppresses sperm production, and it can do so in a man who never raised the subject because he came in about energy or libido. Ask every patient whether he may want children, document the answer, and refer rather than treat if he does - KORB does not currently offer the adjuncts that preserve fertility alongside therapy.

Pricing and charge codes

Item	Price	Charge code
Initial TRT labs	\$99	FITTRTQ100
Quarterly TRT - visit, medication, shipping	\$399	FITTRTQ001

KORB does not accept insurance.

Pharmacy, states and who may prescribe

TRT is offered in these states only. Testosterone is Schedule III and controlled-substance licensure is far narrower than a pharmacy general shipping footprint, so a pharmacy that fills peptides into a state cannot necessarily fill testosterone there.

State	Pharmacy	Prescriber	Ships the injection kit
California (CA)	Empower Pharmacy	Larisa Hammond, NP or LaTonya King, DNP	Yes

State	Pharmacy	Prescriber	Ships the injection kit
Texas (TX)	Premier Pharmacy	Don Stevenson, PA-C	Yes

Testosterone may only be prescribed in a state by the provider named for it. If that is not you, do not send the prescription. Route the patient to a provider registered in that state.

Dose ladder

Start at 120 mg per week. Every step is exact on all three routes at 200 mg/mL, so the weekly dose in this table is the weekly dose in the patient regardless of route. All volumes are read in mL on a Luer lock barrel.

Weekly dose	IM once weekly (3 mL Luer lock)	SQ twice weekly (1 mL Luer lock)	SQ three times weekly (1 mL Luer lock)	Vial lasts	Days written
120 mg	0.60 mL	0.30 mL	0.20 mL	116 days	90 days
144 mg	0.72 mL	0.36 mL	0.24 mL	97 days	90 days
168 mg	0.84 mL	0.42 mL	0.28 mL	83 days	83 days

Testosterone cypionate is a viscous oil: draw up with the wider needle, then change to the finer needle to inject. Do not use insulin syringes, which have a fixed needle and are marked in units. All volumes are read in mL on a Luer lock barrel.

Where the vial lasts longer than the days written, the prescription stops at 90 days and the patient discards the remainder when the next supply arrives. Never tell a patient the vial lasts longer than 90 days.

Use when a patient on twice weekly has low SHBG, trough symptoms, or rising estradiol or hematocrit. It is not a starting regimen. Document the reason for the frequency in the note.

Prescribing - Premier Pharmacy

One complete entry per dose and route. The Name field encodes both, so each combination is its own Tebra favorite and is written out in full rather than factored - a provider copies a whole block into Tebra.

ASK ABOUT FERTILITY BEFORE YOU PRESCRIBE. Testosterone suppresses sperm production, and it can do so in a man who never raised the subject because he came in about energy or libido. Ask every patient whether he may want children, document the answer, and refer rather than treat if he does - KORB does not currently offer the adjuncts that preserve fertility alongside therapy.

Premier Pharmacy · Texas (TX). Testosterone may only be prescribed in a state by the provider named for it. If that is not you, do not send the prescription. Route the patient to a provider registered in that state. In Texas: **Don Stevenson, PA-C**.

Premier Pharmacy · Tebra Standard prescription · 120 mg/week - IM once weekly
90-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	PREMIER - Testosterone 120 mg - IM Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	90
Patient Instructions	Inject 0.60 ml (120 mg) IM weekly or as directed by the physician. 90 day supply. Discard unused portion after 90 days.
Pharmacy Instructions	Alc pads x13; 3mL LL syr x13; 23G 1in x13 (draw); 25G 1in x13 (inject), Bill to office/ship to patient

Premier Pharmacy • Tebra Standard prescription • 120 mg/week - SQ twice weekly

90-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	PREMIER - Testosterone 120 mg - SQ 2x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	90
Patient Instructions	Inject 0.30 ml (60 mg) SQ two times a week or as directed by the physician. 90 day supply. Discard unused portion after 90 days.
Pharmacy Instructions	Alc pads x26; 1mL LL syr x26; 23G 1in x26 (draw); 27G 1/2in x26 (inject), Bill to office/ship to patient

Premier Pharmacy • Tebra Standard prescription • 120 mg/week - SQ three times weekly

90-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	PREMIER - Testosterone 120 mg - SQ 3x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	90
Patient Instructions	Inject 0.20 ml (40 mg) SQ three times a week or as directed by the physician. 90 day supply. Discard unused portion after 90 days.
Pharmacy Instructions	Alc pads x39; 1mL LL syr x39; 23G 1in x39 (draw); 27G 1/2in x39 (inject), Bill to office/ship to patient

Premier Pharmacy · Tebra Standard prescription · 144 mg/week - IM once weekly		90-DAY SUPPLY
Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.	
Name	PREMIER - Testosterone 144 mg - IM Weekly	
Allow Substitution	Yes - select Allow Substitution	
Quantity	10	
Unit	ml	
Refill	0	
Days Supply	90	
Patient Instructions	Inject 0.72 ml (144 mg) IM weekly or as directed by the physician. 90 day supply. Discard unused portion after 90 days.	
Pharmacy Instructions	Alc pads x13; 3mL LL syr x13; 23G 1in x13 (draw); 25G 1in x13 (inject), Bill to office/ship to patient	

Premier Pharmacy · Tebra Standard prescription · 144 mg/week - SQ twice weekly		90-DAY SUPPLY
Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.	
Name	PREMIER - Testosterone 144 mg - SQ 2x Weekly	
Allow Substitution	Yes - select Allow Substitution	
Quantity	10	
Unit	ml	
Refill	0	
Days Supply	90	
Patient Instructions	Inject 0.36 ml (72 mg) SQ two times a week or as directed by the physician. 90 day supply. Discard unused portion after 90 days.	
Pharmacy Instructions	Alc pads x26; 1mL LL syr x26; 23G 1in x26 (draw); 27G 1/2in x26 (inject), Bill to office/ship to patient	

Premier Pharmacy • Tebra Standard prescription • 144 mg/week - SQ three times weekly

90-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	PREMIER - Testosterone 144 mg - SQ 3x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	90
Patient Instructions	Inject 0.24 ml (48 mg) SQ three times a week or as directed by the physician. 90 day supply. Discard unused portion after 90 days.
Pharmacy Instructions	Alc pads x39; 1mL LL syr x39; 23G 1in x39 (draw); 27G 1/2in x39 (inject), Bill to office/ship to patient

Premier Pharmacy • Tebra Standard prescription • 168 mg/week - IM once weekly

83-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	PREMIER - Testosterone 168 mg - IM Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	83
Patient Instructions	Inject 0.84 ml (168 mg) IM weekly or as directed by the physician. 83 day supply.
Pharmacy Instructions	Alc pads x12; 3mL LL syr x12; 23G 1in x12 (draw); 25G 1in x12 (inject), Bill to office/ship to patient

Premier Pharmacy • Tebra Standard prescription • 168 mg/week - SQ twice weekly

83-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	PREMIER - Testosterone 168 mg - SQ 2x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	83
Patient Instructions	Inject 0.42 ml (84 mg) SQ two times a week or as directed by the physician. 83 day supply.
Pharmacy Instructions	Alc pads x24; 1mL LL syr x24; 23G 1in x24 (draw); 27G 1/2in x24 (inject), Bill to office/ship to patient

Premier Pharmacy • Tebra Standard prescription • 168 mg/week - SQ three times weekly

83-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	PREMIER - Testosterone 168 mg - SQ 3x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	83
Patient Instructions	Inject 0.28 ml (56 mg) SQ three times a week or as directed by the physician. 83 day supply.
Pharmacy Instructions	Alc pads x36; 1mL LL syr x36; 23G 1in x36 (draw); 27G 1/2in x36 (inject), Bill to office/ship to patient

Prescribing - Empower Pharmacy

Empower ships the injection kit with the vial, so the pharmacy instructions carry the supply counts. That is the only difference from the Premier entries above.

ASK ABOUT FERTILITY BEFORE YOU PRESCRIBE. Testosterone suppresses sperm production, and it can do so in a man who never raised the subject because he came in about energy or libido. Ask every patient whether he may want children, document the answer, and refer rather than treat if he does - KORB does not currently offer the adjuncts that preserve fertility alongside therapy.

Empower Pharmacy · California (CA). Testosterone may only be prescribed in a state by the provider named for it. If that is not you, do not send the prescription. Route the patient to a provider registered in that state. In California: **Larisa Hammond, NP or LaTonya King, DNP**.

Empower Pharmacy · Tebra Standard prescription · 120 mg/week - IM once weekly

90-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	EMPOWER - Testosterone 120 mg - IM Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	90
Patient Instructions	Inject 0.60 ml (120 mg) IM weekly or as directed by the physician. 90 day supply. Discard unused portion after 90 days.
Pharmacy Instructions	Alc pads x13; 3mL LL syr x13; 23G 1in x13 (draw); 25G 1in x13 (inject), Bill to office/ship to patient

Empower Pharmacy · Tebra Standard prescription · 120 mg/week - SQ twice weekly
90-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	EMPOWER - Testosterone 120 mg - SQ 2x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	90
Patient Instructions	Inject 0.30 ml (60 mg) SQ two times a week or as directed by the physician. 90 day supply. Discard unused portion after 90 days.
Pharmacy Instructions	Alc pads x26; 1mL LL syr x26; 23G 1in x26 (draw); 27G 1/2in x26 (inject), Bill to office/ship to patient

Empower Pharmacy · Tebra Standard prescription · 120 mg/week - SQ three times weekly
90-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	EMPOWER - Testosterone 120 mg - SQ 3x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	90
Patient Instructions	Inject 0.20 ml (40 mg) SQ three times a week or as directed by the physician. 90 day supply. Discard unused portion after 90 days.
Pharmacy Instructions	Alc pads x39; 1mL LL syr x39; 23G 1in x39 (draw); 27G 1/2in x39 (inject), Bill to office/ship to patient

Empower Pharmacy · Tebra Standard prescription · 144 mg/week - IM once weekly
90-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	EMPOWER - Testosterone 144 mg - IM Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	90
Patient Instructions	Inject 0.72 ml (144 mg) IM weekly or as directed by the physician. 90 day supply. Discard unused portion after 90 days.
Pharmacy Instructions	Alc pads x13; 3mL LL syr x13; 23G 1in x13 (draw); 25G 1in x13 (inject), Bill to office/ship to patient

Empower Pharmacy · Tebra Standard prescription · 144 mg/week - SQ twice weekly
90-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	EMPOWER - Testosterone 144 mg - SQ 2x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	90
Patient Instructions	Inject 0.36 ml (72 mg) SQ two times a week or as directed by the physician. 90 day supply. Discard unused portion after 90 days.
Pharmacy Instructions	Alc pads x26; 1mL LL syr x26; 23G 1in x26 (draw); 27G 1/2in x26 (inject), Bill to office/ship to patient

Empower Pharmacy · Tebra Standard prescription · 144 mg/week - SQ three times weekly
90-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	EMPOWER - Testosterone 144 mg - SQ 3x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	90
Patient Instructions	Inject 0.24 ml (48 mg) SQ three times a week or as directed by the physician. 90 day supply. Discard unused portion after 90 days.
Pharmacy Instructions	Alc pads x39; 1mL LL syr x39; 23G 1in x39 (draw); 27G 1/2in x39 (inject), Bill to office/ship to patient

Empower Pharmacy · Tebra Standard prescription · 168 mg/week - IM once weekly
83-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	EMPOWER - Testosterone 168 mg - IM Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	83
Patient Instructions	Inject 0.84 ml (168 mg) IM weekly or as directed by the physician. 83 day supply.
Pharmacy Instructions	Alc pads x12; 3mL LL syr x12; 23G 1in x12 (draw); 25G 1in x12 (inject), Bill to office/ship to patient

Empower Pharmacy · Tebra Standard prescription · 168 mg/week - SQ twice weekly
83-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	EMPOWER - Testosterone 168 mg - SQ 2x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	83
Patient Instructions	Inject 0.42 ml (84 mg) SQ two times a week or as directed by the physician. 83 day supply.
Pharmacy Instructions	Alc pads x24; 1mL LL syr x24; 23G 1in x24 (draw); 27G 1/2in x24 (inject), Bill to office/ship to patient

Empower Pharmacy · Tebra Standard prescription · 168 mg/week - SQ three times weekly
83-DAY SUPPLY

Drug	Testosterone Cypionate 200mg per mL inj (commercial generic) Select this from the Tebra drop-down. Do not copy and paste.
Name	EMPOWER - Testosterone 168 mg - SQ 3x Weekly
Allow Substitution	Yes - select Allow Substitution
Quantity	10
Unit	ml
Refill	0
Days Supply	83
Patient Instructions	Inject 0.28 ml (56 mg) SQ three times a week or as directed by the physician. 83 day supply.
Pharmacy Instructions	Alc pads x36; 1mL LL syr x36; 23G 1in x36 (draw); 27G 1/2in x36 (inject), Bill to office/ship to patient

Standard lab panel

Select the KORB TRT panel in the Quest integration in Tebra. The panel is already built; the tests below are listed for reference and are NOT ordered individually.

Full panel at baseline and at each follow-up. Recheck 6 to 8 weeks after any change.

Test	Quest code
FSH & LH	7137
PSA, Total	5363
Prolactin	746
CBC	1759
Estradiol	4021
Testosterone, Total, MS	15983

Draw in the MORNING, FASTING, before 10:00 - the KORB standard for all lab draws, and the window the Men's Health SOP specifies for testosterone (7 to 10 AM). Draw testosterone at trough, on an injection day before the dose is given. A peak draw will read high and hide a symptomatic trough.

Add-on labs

Ordered individually when clinically indicated, not part of the KORB TRT panel. Same draw conditions as the standard panel.

Lab	Quest code	Charge code	Price
Testosterone, Free (Dialysis) and Total, LC/MS/MS	37073	KLAB37073	\$275
Sex Hormone-Binding Globulin (SHBG)	30740	KLAB30740	\$105
Comprehensive Metabolic Panel	10231	KLAB10231	\$45
Lipid Panel	7600	KLAB7600	\$95
Hemoglobin A1c	496	KLAB496	\$55
Thyroid Panel with TSH, Free T3, Free T4	34429	KLAB34429	\$165
Vitamin D, 25-Hydroxy, Total, Immunoassay	17306	KLAB17306	\$185
DHEA Sulfate	402	KLAB402	\$155
C-Reactive Protein, Cardiac (High Sensitivity)	10124	KLAB10124	\$75

Lab	Quest code	Charge code	Price
Ferritin	457	KLAB457	\$85

Providers who order optional or add-on labs are responsible for reviewing, addressing and documenting ALL results, including abnormalities not directly related to TRT. Ordering additional labs expands the clinical scope of the encounter and may uncover comorbidities that require counselling, follow-up or referral. Order them when clinically indicated and when you are prepared to manage whatever comes back.

Monitoring timeline

When	Labs	Purpose
Baseline, before TRT	TT, E2, LH/FSH, CBC, PSA, prolactin	Establish the diagnosis, confirm eligibility, identify contraindications.
4 to 6 weeks after starting	TT, E2, LH/FSH, CBC, PSA, prolactin	Initial therapeutic response, erythrocytosis risk, dose accuracy.
Every 12 weeks	TT, E2, CBC, drawn 48 hours post-injection	Therapeutic response, erythrocytosis risk, dose accuracy.
Annually	Full panel - TT, E2, LH/FSH, CBC, PSA, prolactin	Full annual review.
As indicated	Any of the above, plus add-ons	Dose changes, side effects, or abnormal prior results.

PSA applies to men 40 and older or with risk factors. Recheck 6 to 8 weeks after any dose or frequency change, drawn at trough.

Titration targets and action thresholds

Marker	Threshold	Action
Total testosterone (trough)	Goal band 400 - 700 ng/dL	In band with symptoms resolved: hold current dose.
Total testosterone (trough)	Below 400 ng/dL with persistent symptoms	Move up one step on the ladder. Recheck in 6 to 8 weeks.

Marker	Threshold	Action
Total testosterone (trough)	Above 700 ng/dL	Move down one step, even if the patient feels well.
Total testosterone (trough)	Above 1000 ng/dL	Reduce dose regardless of symptoms. Supraphysiologic levels carry cardiovascular and polycythemia risk.
Hematocrit	50 - 53%	Warning range. Recheck, review sleep apnea and hydration, consider increasing injection frequency.
Hematocrit	Above 54%	Intervene. Hold or reduce dose, increase frequency, and discuss therapeutic phlebotomy. Do not continue unchanged.
Estradiol	Elevated with symptoms (gynecomastia, nipple tenderness, mood, fluid retention)	Increase injection frequency first. Reduce dose if that is insufficient. An aromatase inhibitor is a last resort.
PSA	Rise greater than 1.4 ng/mL within 12 months, or absolute above 4.0 ng/mL	Refer to urology before continuing therapy.

Change one variable at a time. Adjusting dose and frequency together makes it impossible to know which one worked. Recheck labs 6 to 8 weeks after any change, drawn at trough.

Hematologic safety and phlebotomy

Erythrocytosis is the commonest reason to stop testosterone. More frequent smaller injections lower the peak and often resolve it without a dose reduction.

- Hold or reduce the dose if hematocrit reaches 54%.
- Require documented blood donation or therapeutic phlebotomy before restarting.
- Resume only after hematocrit is below 52%, with the intervention documented in the EHR.
- If hematocrit stays elevated despite intervention, refer to hematology.
- Above 52% on therapy, re-evaluate for dehydration or dosing interval before assuming the dose is the cause.

Prostate safety and PSA

Obtain a baseline PSA for all men, and particularly those 40 and older or with risk factors. Use age-adjusted ranges.

Age	Age-adjusted PSA range	Action above the range
40 - 49	0.0 - 2.5 ng/mL	Above 2.5, evaluate family history and risk.
50 - 59	0.0 - 3.5 ng/mL	Above 3.5, consider urology referral.
60 - 69	0.0 - 4.5 ng/mL	Above 4.5, further diagnostic review.
70 and older	0.0 - 6.5 ng/mL	Evaluate in the context of comorbidities.

Hold TRT and refer to urology if PSA is above 4.0 ng/mL, or above 3.0 in a high-risk group, or has risen more than 1.4 ng/mL in 12 months. Resume only after urology clearance and a documented negative malignancy evaluation.

Cardiovascular and metabolic safety

- Do not begin TRT until blood pressure is controlled, below 140/90. This is harder over telemedicine - confirm the PCP or specialist is managing it and that it is in range.
- Screen for sleep apnea and obesity-related hypoventilation by referring the patient to their PCP.
- Assess HbA1c and a lipid panel for metabolic syndrome.
- Coordinate with the PCP or cardiology for known ASCVD or diabetes.
- Emphasise lifestyle optimisation - nutrition, exercise, stress reduction.

Side effects and counselling

System	Effect	Counselling and action
Hematologic	Elevated hematocrit	Increase hydration; consider dose adjustment, more frequent smaller injections, or phlebotomy if hematocrit reaches 54%.
Endocrine	Elevated estradiol	Review for mood change, bloating and gynecomastia. Treat only if symptomatic.
Dermatologic	Acne, oily skin, hair thinning	Advise skin hygiene; avoid over-supplementation such as DHEA.
Mood and CNS	Irritability, mood swings	Assess estradiol and stress. Avoid abrupt dose increases.
Cardiovascular	Fluid retention, raised blood pressure	Monitor blood pressure; ensure hydration and reduce sodium.
Genitourinary	Reduced fertility, testicular atrophy	Discuss fertility preservation. See Fertility - KORB does not currently offer the adjuncts.

Document the symptom review, the education given and the plan for any side effect the patient reports.

Controlled substance workflow

Timing is computed per patient by the TRT Provider Tool. The rules it applies are these.

- Check the patient prescription-monitoring database in THEIR state before every controlled-substance prescription, not just the first. It is how you find a patient receiving testosterone or other controlled substances from another prescriber. Record that you checked, and the last fill date, in the visit note.
- Texas: Texas Prescription Monitoring Program (PMP Aware).
- California: CURES - Controlled Substance Utilization Review and Evaluation System. CONFIRM this before the first California prescription - it has not been checked against a KORB source.
- Refills count from the PMP last fill date, not the visit date.
- Order labs 14 days before the next visit. Order labs 14 days before the next visit. Results can take a week, so the draw must not be left to the final days.
- Send the prescription 4 days before the pharmacy can release it. Send the prescription 4 days before the pharmacy can release it. Earlier is what triggers the too-early callback.
- Nothing is dispensed or scheduled beyond 90 days. Nothing is dispensed or scheduled beyond 90 days. At 120 and 144 mg per week the vial physically outlasts 90 days and the remainder is discarded; at 168 it runs out at 83 days and the prescription is written for 83. Never tell a patient the vial lasts longer than 90 days.
- Review the PMP at every visit and record the last fill date in the note.

Do not initiate - absolute contraindications

Do not initiate testosterone therapy with any of the following. Refer to PCP, urology, or the appropriate specialist.

- Known or suspected prostate or breast cancer
- Hematocrit above 52% at baseline - hold and evaluate for erythrocytosis, dehydration or sleep apnea
- Severe untreated obstructive sleep apnea
- Uncontrolled heart failure; MI or stroke within the last 6 months
- Thrombophilia
- Active desire for fertility WITHOUT concurrent gonadotropin or enclomiphene support - KORB does not currently offer either, so this means refer
- PSA above 4.0 ng/mL, or above 3.0 ng/mL with high prostate cancer risk, without urological evaluation first

Precautions - settle these first

Relative contraindications. These are not absolute bars, but each needs addressing before therapy rather than after.

- Polycythemia, or hematocrit above 50% - re-evaluate for sleep apnea, dehydration or high-dose therapy.
- Prostate enlargement (BPH) - monitor urinary symptoms and the PSA trend.
- Severe lower urinary tract symptoms (AUA score above 19) - refer to urology first.
- Untreated thyroid dysfunction, metabolic syndrome or severe obesity - optimise first.
- Psychiatric instability or uncontrolled mood disorder - stabilise first, and coordinate with the PCP.
- High cardiovascular risk or active atherosclerosis - obtain clearance from the PCP or a cardiologist.

Stopping therapy

TRT may be discontinued under supervision when any of the following applies. Stopping is a clinical event with its own follow-up, not simply the absence of a prescription.

- Hematocrit persistently above 54%.
- PSA elevation not cleared by urology.
- Patient preference, or side effects that outweigh the benefit.
- Endogenous testosterone may take 6 to 12 weeks to return to normal.
- Taper gradually rather than stopping abruptly, or transition to enclomiphene to support recovery, noting that KORB does not currently offer it.
- Schedule a follow-up telemedicine visit 6 to 8 weeks after stopping, to review labs and symptoms.
- If the patient declines recommended labs, phlebotomy or a follow-up visit, document the refusal and the counselling on the associated risks.

Questions and escalation

- Pharmacy or shipping issues — Operations
- Charge codes and billing — Clinical Operations
- Clinical protocol questions — Clinical Operations
- Corrections to this document — Clinical Operations

Route everything through Clinical Operations. Do not contact the clinical sign-off physician directly; Clinical Operations escalates when it needs to.

Do not improvise

- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not cross a gender-specific product
- Do not quote pricing to a patient without confirming with Operations

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